

Health and the African Diaspora

Passing health knowledge from prevention to medication: food, lifestyle, laboratory testing, daily routine, and responsible care.



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Editorial Note

This publication layout preserves the original essay while presenting it in a more polished editorial format for sharing, printing, or public discussion. The symbol combines a bridge, an African health silhouette, a cross, and a leaf to represent diaspora connection, preventive care, and food as the foundation of health.

Do not only send money for sickness; send knowledge for health.

Central message of the essay

Core Idea	Publishing Treatment
Health begins before medication.	Highlighted through cover subtitle, pull quotes, and a preventive-care table.
The diaspora has know-how.	Presented as a bridge of knowledge, mentorship, research, investment, and culture.
Africa needs prevention systems.	Structured through food, lifestyle, laboratory testing, daily routine, and responsible medication.

Introduction: Health Is More Than Medication

Health is often discussed only when sickness has already arrived. In many communities, the word “healthcare” is understood as visiting a clinic, receiving a diagnosis, buying medication, or going to the hospital when the body can no longer cope. Yet this is only the final part of health.

True health begins much earlier. It begins with **food, lifestyle, clean environments, laboratory testing, daily routine, mental balance, community education, and prevention**. Medication is important, but medication should not be the first language of health. It should be the last line of defense after prevention, early detection, and daily discipline have done their work.

This is where the African diaspora has a historic role. Across Europe, North America, the Middle East, and other parts of the world, Africans in the diaspora have gained health knowledge through professional training, workplace systems, insurance systems, health technology, research institutions, nutrition awareness, laboratory access, preventive screening, and public health culture. The question is no longer whether the diaspora has knowledge. The deeper question is whether the diaspora is **passing health**, not only sending money.

The World Health Organization defines primary health care as a “whole-of-society approach” that focuses on people’s needs as early as possible, from health promotion and disease prevention to treatment, rehabilitation, and palliative care.

World Health Organization [1]

That definition confirms the central argument of this essay: **healthcare is not only medicine; healthcare is prevention organized into daily life**. For the African continent, the diaspora can help solve the health puzzle by shifting attention from emergency treatment to preventive living, from imported solutions to locally adapted knowledge, and from individual charity to organized systems of health education and community transformation.

The Health Puzzle in Africa

Africa faces a complex health puzzle because the continent must address infectious diseases, maternal and child health needs, undernutrition, mental health challenges, environmental risks, and the rising burden of noncommunicable diseases such as diabetes, hypertension, heart disease, stroke, cancers, and chronic respiratory conditions. These challenges do not come from one cause, so they cannot be solved by medication alone.

The African health puzzle also includes a knowledge gap. Many people know what to do only after sickness becomes severe, but fewer people are regularly taught how to prevent illness before it begins. A family may spend heavily on hospital bills but may not have a routine culture of annual laboratory tests, dental care, eye checks, reproductive health screening, or early cancer screening where available.

Dimension of Health	Common Late Response	Preventive Foundation Needed
Food	Treating diabetes, obesity, ulcers, or hypertension after diagnosis.	Nutrition education, local healthy foods, portion control, reduced ultra-processed foods, and lower salt and sugar intake.
Lifestyle	Medication after disease progresses.	Regular movement, sleep discipline, stress management, and reduced tobacco and harmful alcohol use.
Laboratory testing	Testing only when seriously ill.	Routine blood pressure checks, blood sugar tests, cholesterol tests, kidney and liver function tests, HIV testing, malaria testing when symptomatic, and age-appropriate screening.
Daily routine	Health ignored until pain appears.	Daily hydration, hygiene, activity, balanced meals, rest, mental care, and medication adherence when prescribed.
Medication	Self-medication or late hospital treatment.	Safe prescribing, correct diagnosis, pharmacist guidance, adherence, and avoidance of misuse.

The diaspora can help Africa move from the left column to the right column. This does not mean imposing foreign habits on African communities. It means translating useful knowledge into African realities: local foods, local languages, local beliefs, local markets, local schools, local churches and mosques, local clinics, and local digital platforms.

The Diaspora Has Know-How, But Is It Passing Health?

The African diaspora has long contributed to the continent through remittances, business investment, education, emergency relief, and family support. These contributions matter. However, health requires more than money. Health requires **knowledge transfer, behavior change, institutional partnership, and trust.**

A relative abroad may send money for hospital bills, but the more transformative act is to help the family understand why blood pressure must be checked regularly, why laboratory tests matter before symptoms become severe, why children need balanced nutrition, why women's health screening saves lives, and why men must not wait until crisis before seeking care.

Diaspora engagement in health is not theoretical. The International Organization for Migration has documented diaspora health projects in Africa in which health professionals abroad supported hospitals, universities, training institutions, and public health systems through temporary return, training, and knowledge sharing. In Ghana, the MIDA Ghana Health project reported 282 assignments since 2005, 17 internships for Ghana-based health professionals in Europe, and more than 21,000 Ghanaian health workers and students directly benefiting from capacity-building initiatives by more than 100 Europe-based Ghanaian diaspora health professionals [2].

This example shows that diaspora knowledge can travel. But the next stage must be broader. The diaspora should not only train doctors and nurses. It should also train families, schools, churches, mosques, farmers, market women, youth groups, sports clubs, media houses, and local leaders.

Rebuilding the Meaning of Health: Food First

Food is the first medicine, not because food replaces medical treatment, but because food shapes the body every day. The continent has rich food traditions: vegetables, beans, lentils, millet, sorghum, teff, plantain, yam, cassava, fish, nuts, fruits, spices, fermented foods, and many forms of local grains.

Yet urbanization and global consumer culture have increased dependence on refined carbohydrates, sugary drinks, excessive salt, deep-fried foods, oversized portions, and ultra-processed products. The result is a quiet rise in obesity, diabetes, hypertension, kidney disease, and heart disease.

The diaspora can help by promoting nutrition literacy without insulting African food. The goal is not to make Africa eat like Europe or America. The goal is to help Africans understand how to use African foods wisely. Diaspora nutritionists, nurses, doctors, public health professionals, chefs, fitness educators, and community organizers can create simple guides in local languages showing how to build balanced plates from local foods.

Food education should be practical. People need to know what to eat before diabetes appears, not only what to avoid after diabetes is diagnosed.

Lifestyle: The Daily Routine Is the Hidden Clinic

The second foundation is lifestyle. A clinic may see a patient for twenty minutes, but a person lives with their own habits twenty-four hours a day. Daily routine is therefore a hidden clinic. Sleep, movement, hygiene, stress, social connection, work conditions, alcohol, tobacco, screen time, sexual health, and mental health all become part of the health system.

The diaspora can help normalize preventive routines. In many countries where Africans live abroad, annual checkups, blood pressure machines, fitness centers, mental health conversations, patient reminders, food labels, pharmacy consultations, and workplace wellness programs are more visible. The diaspora can adapt these habits for the continent.

A village does not need a luxury gym to promote movement. Walking groups, school sports, dance, farming activity, community clean-up days, and workplace stretching can be health interventions. A family does not need expensive technology to promote sleep discipline, reduced late-night eating, or daily hydration.

Laboratory Tests and Early Detection: Knowing Before It Is Too Late

The third foundation is laboratory testing and screening. Many deaths occur not because disease is untreatable, but because disease is discovered too late. Hypertension may be silent. Diabetes may develop quietly. Kidney disease may progress before symptoms become obvious. Some infections can be treated effectively when detected early. Laboratory tests and screening are therefore not luxuries; they are tools of prevention.

WHO lists disease prevention and early detection, public health surveillance, health promotion, community engagement, workforce development, and access to health products and technologies among essential public health functions [3]. This matters because a preventive health system must know what is happening in the population. It must measure, test, monitor, and respond early.

The diaspora can support laboratory capacity by partnering with clinics and universities, training laboratory scientists and biomedical engineers, supporting affordable diagnostic centers, designing community screening days, and improving digital reminders, patient records, and referral systems.

Medication: Necessary, But Not the Foundation

Medication is essential when disease requires treatment. A responsible essay on prevention must not create the false impression that lifestyle alone can cure every illness. Antibiotics, antimalarials, insulin, blood pressure medicines, vaccines, cancer treatments, HIV medicines, pain management, emergency surgery, and many other interventions save lives. The problem is not medication itself. The problem is making medication the beginning and end of healthcare.

When medication becomes the only health strategy, people may self-medicate, misuse antibiotics, delay diagnosis, stop treatment early, or depend on pills while ignoring food, exercise, sleep, and follow-up tests. Pharmacists and clinicians in the diaspora can train community pharmacists, support antimicrobial resistance education, explain adherence, and create culturally relevant materials about when to seek professional care.

Medication should sit inside a larger health ladder: food, lifestyle, testing, routine, clinical guidance, and then medicine when needed.

From Brain Drain to Brain Circulation

For decades, African health migration has been described as “brain drain,” because skilled doctors, nurses, scientists, pharmacists, and researchers leave the continent. This concern is real, but the story does not have to end with loss. It can become **brain circulation**.

Brain circulation means that knowledge, skills, research, mentorship, technology, and investment move back and forth between the diaspora and the continent. A surgeon abroad can mentor residents online. A nurse can help develop infection-prevention protocols. A nutritionist can create local-language food education. A laboratory scientist can train technicians on quality control. A software engineer can build health reminder systems. A diaspora association can sponsor screening campaigns and health education.

The African Diaspora Network describes the diaspora as a catalyst bringing technical expertise, mentorship, capital, and global networks, while emphasizing that Africa carries nearly a quarter of the global disease burden but produces only about 2% of health research and attracts around 1% of global research and development investment [4]. This gap is not only a statistic; it is a call to responsibility.

A Practical Framework: How the Diaspora Can Solve the Puzzle

The diaspora can solve the preventive health puzzle only if it moves from scattered goodwill to organized action. Individual help is valuable, but systems change requires structure.

1. Health educator

Create local-language content on food, blood pressure, diabetes, maternal health, mental health, and screening.

2. Mentor and trainer

Link diaspora professionals with African clinics, universities, laboratories, and pharmacies.

3. Investor and innovator

Support affordable diagnostics, telehealth, community pharmacies, local food systems, and health data tools.

4. Community bridge

Work with hometown associations, faith groups, schools, and traditional leaders to build trust.

5. Policy advocate

Support public health policy, food standards, laboratory quality, insurance, and primary care financing.

First, the diaspora should build health literacy campaigns. Second, diaspora associations should adopt communities, not just events. Third, the diaspora should invest in prevention infrastructure. Fourth, diaspora professionals must practice humility. Fifth, governments and institutions on the continent must create channels for diaspora contribution.

The Moral Responsibility of Passing Health

The diaspora's responsibility is not only professional; it is moral. Many Africans abroad have seen health systems where prevention is more organized, where laboratory tests are routine, where food labels are visible, where screening is encouraged, and where public health messages are repeated. Not every country abroad has a perfect system, and many diaspora communities also suffer from inequality and disease. Yet exposure to different systems gives the diaspora a comparative advantage. That advantage should be shared.

Passing health means calling home not only to ask about school fees and rent, but also to ask: Have you checked your blood pressure? When was your last blood sugar test? Are the children eating vegetables? Is grandmother taking her medicine correctly? Has the pregnant mother attended antenatal care? Is the family reducing salt and sugary drinks? Is anyone silently struggling with depression? Are we waiting for sickness, or are we building health every day?

Conclusion: From Sending Money to Sending Health

The African diaspora has already proven its love for the continent through remittances, advocacy, education, and emergency support. The next stage is to send health in a deeper sense. Sending health means transferring knowledge, building systems, supporting prevention, strengthening laboratories, mentoring health workers, investing in local solutions, and teaching families that the first hospital is the daily routine.

Africa's health future will not be secured by medication alone. Medication treats disease, but prevention protects destiny. Food, lifestyle, laboratory testing, daily routine, and responsible medical care must become one connected chain. The diaspora can help forge that chain by turning brain drain into brain circulation, charity into capacity, and advice into organized community action.

The puzzle can be solved if the diaspora understands its role clearly: do not only send money for sickness; send knowledge for health.

References

[1] World Health Organization, “Primary health care.”

[2] International Organization for Migration, “Diaspora engagement projects in the health sector in Africa.”

[3] World Health Organization, “Essential public health functions.”

[4] African Diaspora Network, “Why diaspora science matters for strengthening Africa’s health and innovation ecosystem.”

Design note: The symbol used in this publication represents Africa, prevention, and the diaspora knowledge bridge.